

Sheffield Teaching Hospitals — Renal Outpatient Department

Date: 14 May 2024

CLINIC LETTER

Patient: Christopher Walsh

DOB: 1959-09-27 (age 64)

NHS Number: 999 130 0013

Dear Dr Henderson,

Thank you for referring Mr Walsh for renal assessment following his recent decline in kidney function. I reviewed him today in the low-clearance clinic.

History: CKD known since 2021, previously stage 3a. Presented to GP in late March with fatigue and reduced exercise tolerance over 8 weeks. No haematuria, no flank pain, no recent NSAID use, no IV contrast exposure. Adherent with ramipril and other medications. No recent infections.

Examination: BP 156/88, HR 76. Weight 83 kg. Trace ankle oedema bilaterally. JVP not elevated. Chest clear. No bruits.

Investigations (today, repeated from GP April panel): eGFR 28 mL/min/1.73m² (down from 52 in January); creatinine 198 µmol/L (up from 128); urea 14.2; potassium 5.1 (upper limit); bicarbonate 19 (mild metabolic acidosis); haemoglobin 108 g/L (anaemia of CKD); phosphate 1.42 (mildly elevated); calcium 2.28 (adjusted, low-normal); PTH 11.2 pmol/L (elevated); ACR 22 mg/mmol (now A3, moderate albuminuria); renal ultrasound: bilateral parenchymal changes consistent with chronic disease, no obstruction.

Allergies: NKDA.

Assessment: CKD has progressed to stage 4 (eGFR 28). Aetiology: diabetic nephropathy + hypertensive nephrosclerosis. Patient now has CKD-MBD picture emerging (elevated PTH, mild hyperphosphataemia) and renal anaemia. Trajectory suggests renal replacement therapy planning within 18-24 months. Discussed implications with patient and his wife at length.

Plan:

- **Reclassification: CKD stage 4** — please update practice record
- Continue ramipril 10 mg OD — monitor potassium closely; reduce if K⁺>5.5
- Hold metformin and review for alternative agent given eGFR 28 (close to contraindication threshold of 30)
- Pneumococcal vaccination today; ensure annual flu and 5-yearly Hep B series initiated
- Dietetics referral — low potassium, low phosphate dietary education
- Anaemia workup: iron studies, B12/folate; consider ESA if Hb <100
- Initial discussion of vascular access (AV fistula) given trajectory — not for action yet but for planning

- Repeat bloods in 6 weeks at GP
- Renal clinic follow-up in 3 months

Yours sincerely,

Dr R. Patel, Consultant Nephrologist